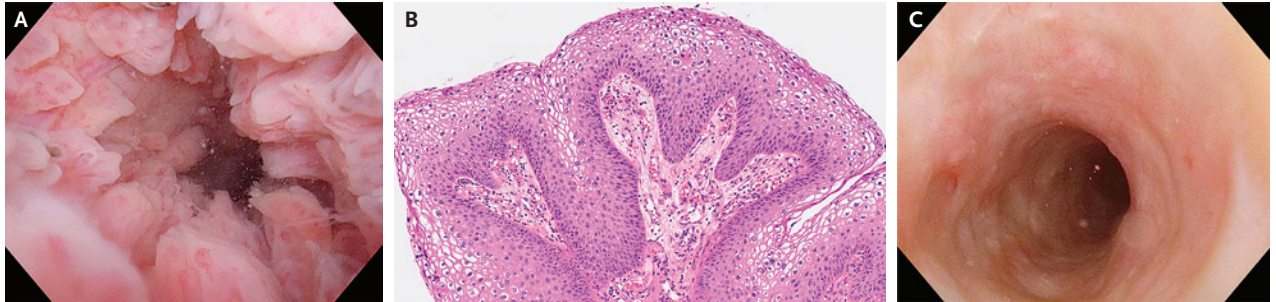


IMAGES IN CLINICAL MEDICINE

Condyloma Acuminata of the Urethra



A 42-YEAR-OLD MAN PRESENTED TO THE UROLOGY CLINIC WITH A 2-WEEK history of urinary frequency and weak stream. Four months before presentation, the patient had undergone stem-cell transplantation for acute lymphoblastic leukemia, which had been complicated by chronic graft-versus-host disease that had led to prolonged immunosuppressive therapy. One month before presentation, the patient had noted multiple small bumps on his penis. He was in a monogamous relationship with one sexual partner. Physical examination was notable for several small papules on the penile shaft. Urinalysis showed 9 white cells per high-power field, and a urine culture was negative. Testing for human immunodeficiency virus was also negative. Urethroscopy showed papillomatous lesions carpeting the urethral mucosa (Panel A). The penile papules were excised and identified on histopathological analysis as condyloma acuminata (genital warts), and resected urethral tissue showed papillomatous squamous proliferation with koilocytic atypia (Panel B, hematoxylin and eosin stain). A polymerase-chain-reaction assay of the excised penile-lesion specimen was positive for human papilloma virus (HPV) type 11 (the patient had not been vaccinated against HPV). A diagnosis of condyloma acuminata of the urethra was made. Treatment consisted of transurethral ablation of the lesions followed by intraurethral instillations of 5-fluorouracil and additional transurethral ablations. At follow-up 11 months after the initial diagnosis, the urethral lesions had not recurred (Panel C).

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